Back Pain

UP-05 | Universal Patient Care

Quick flow

01 Initial Assessment [EMT / AEMT / Paramedic]

Primary survey • vital signs • pain score • relevant history • traumatic or atraumatic onset

02 Focused Neurologic Assessment [EMT / AEMT / Paramedic]

Bilateral leg strength and sensation • distal pulses • gait when safe • ask about saddle anesthesia and bowel/bladder dysfunction

03 New Neurologic Deficit? [EMT / AEMT / Paramedic]

Weakness, sensory loss, saddle anesthesia by history, urinary retention/incontinence, or bowel dysfunction

04 Time-Critical Finding [EMT / AEMT / Paramedic]

Prompt transport • early receiving-facility notification • repeat neurologic examination

05 Treat Pain [EMT / AEMT / Paramedic]

Position of comfort • adult UP-11 or pediatric PM-04 analgesia • cardiac monitoring when medication is administered

06 Reassess + Transport [EMT / AEMT / Paramedic]

Pain score • vital signs • neurologic findings • medication response

Overview

- Assess patients with back pain for trauma and new neurologic dysfunction, provide appropriate analgesia, and reassess the response to treatment.
- Saddle anesthesia is assessed by history. A routine perineal examination is not required in the prehospital setting.

Provider-Level Actions

Higher provider levels include the actions listed for the preceding levels.

EMT

- Obtain the history, pain score, complete vital signs, and a focused examination. Determine whether the pain followed trauma.
- Assess bilateral lower-extremity strength and sensation, distal pulses, and the ability to stand or walk when safe. Ask about new saddle anesthesia, urinary retention or incontinence, and bowel dysfunction. Do not perform a routine perineal examination.
- For traumatic back pain, apply spinal protection when indicated. For uncomplicated atraumatic pain, place the patient in a position of comfort and avoid unnecessary immobilization.
- Provide nonpharmacologic care. For patients age 16 years or older, use UP-11 Adult Pain Control; for patients age 2 through younger than 16 years, use PM-04 Pediatric Pain Control; for patients younger than 2 years, contact Medical Control for analgesic direction. Apply cardiac monitoring when medication is administered.
- Transport promptly and notify the receiving facility early for a new or progressive neurologic deficit, saddle anesthesia, or new bowel or bladder dysfunction.

AEMT

- Establish IV access only when needed for parenteral analgesia, hemodynamic instability, or another specific clinical indication. Do not start an IV or administer fluid routinely for isolated back pain.

- Administer analgesia under the age-appropriate pain pathway, maintain cardiac monitoring after medication administration, and reassess pain, vital signs, neurologic findings, and medication response.

Paramedic

- Interpret the cardiac rhythm and administer parenteral analgesia under UP-11 for patients age 16 years or older or PM-04 for patients age 2 through younger than 16 years when indicated. Continue the medication-specific airway, respiratory, hemodynamic, and cardiac monitoring required by the applicable pathway.
- Acquire a 12-lead ECG or transition to the appropriate cardiac, abdominal, vascular, trauma, or obstetric protocol only when the history, examination, or clinical condition provides a separate indication.

Indications

- Acute or chronic back pain without a more specific immediately apparent treatment pathway.

Assessment | Focused assessment

- Pain onset, location, radiation, severity, provoking or relieving factors, trauma, prior episodes, and relevant medications or anticoagulants.
- Bilateral lower-extremity strength and sensation, distal pulses, and gait when safe.
- Ask about new saddle anesthesia, urinary retention or incontinence, and bowel dysfunction. Saddle anesthesia is a historical symptom; routine prehospital examination of the perineum is not required.

Treatment steps

- Also screen for sudden severe back pain with chest or abdominal pain, syncope, shock, or unequal pulses; fever or infection risk (including immunosuppression, injection drug use, or a recent spinal procedure); cancer history; and pregnancy-related symptoms. These findings require prompt assessment for vascular, infectious, spinal, or obstetric emergencies and transition to the appropriate protocol, even when the initial leg examination is normal.
- Complete the focused neurologic assessment and identify any traumatic mechanism.
- Use spinal protection only when indicated; otherwise position the patient for comfort.
- For patients age 16 years or older, administer analgesia according to UP-11; for patients age 2 through younger than 16 years, use PM-04; for patients younger than 2 years, contact Medical Control. Apply cardiac monitoring when medication is administered.
- Reassess the pain score, vital signs, neurologic examination, and treatment response.
- Transport promptly with early notification for any new or progressive neurologic deficit, saddle anesthesia, or bowel/bladder dysfunction.

Medications

Analgesia

Age 16 years or older: use UP-11 Adult Pain Control. Age 2 through younger than 16 years: use PM-04 Pediatric Pain Control. Younger than 2 years: contact Medical Control.

- Apply cardiac monitoring when analgesic medication is administered.
- Reassess pain, vital signs, neurologic findings, and medication response.

Warnings / precautions

- Do not perform a routine perineal examination solely to assess saddle anesthesia; obtain this finding by history.
- Do not routinely immobilize atraumatic back pain or delay transport for nonessential procedures.
- Do not routinely establish IV access or administer fluid for isolated back pain. Apply cardiac monitoring when analgesic medication is administered; obtain a 12-lead ECG only for a separate clinical indication.

Clinical pearls

- New weakness, sensory loss, saddle anesthesia, urinary retention/incontinence, or bowel dysfunction requires prompt transport and early notification.
- A normal initial neurologic examination does not replace reassessment after analgesia or when symptoms change.

Special populations | Pediatrics - younger than 16 years

- Use PM-04 Pediatric Pain Control for patients age 2 through younger than 16 years and reassess after treatment. Contact Medical Control for analgesic direction for a patient younger than 2 years.

Related pathways / contacts

UP-11 Adult Pain Control - Use for patients age 16 years or older.

PM-04 Pediatric Pain Control - Use for patients age 2 through younger than 16 years.

Source and references

Tennessee EMS BLS/ALS State Protocol Guidelines 2024-2025 - SOP 300 Medical Complaint Not Specified, SOP 608 Spinal Protection, and Pain Management reference | Claiborne County EMS UP-05 source PDF - Back Pain | Claiborne County EMS UP-11 Adult Pain Control and PM-04 Pediatric Pain Control

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